

"I Want My Own Space, but I Want to Be Understood": The Adolescent Emotional Communication Dilemma in Hospitals

Liuping Wang
School of Information Science
Beijing Language and Culture
University
Beijing, China
liupingw@blcu.edu.cn

Mingjing Wu
School of Information Science
Beijing Language and Culture
University
Beijing, China
2012760141@qq.com

Shengmin Zhao
School of Software
Shandong University
Jinan, China
shengmin.zhao@sdu.edu.cn

Wenhui Kang*
Institute of Software
Chinese Academy of Sciences
Beijing, China
ks_moth@163.com

Jin Huang[†]
Institute of Software
Chinese Academy of Sciences
Beijing, China
huangjin@iscas.ac.cn

Chenglei Yang
School of Software
Shandong University
Jinan, China
chl_yang@sdu.edu.cn

Dong Yu
School of Information Science
Beijing Language and Culture
University
Beijing, China
yudong@blcu.edu.cn

Pengyuan Liu
School of Information Science
Beijing Language and Culture
University
Beijing, China
liupengyuan@pku.edu.cn

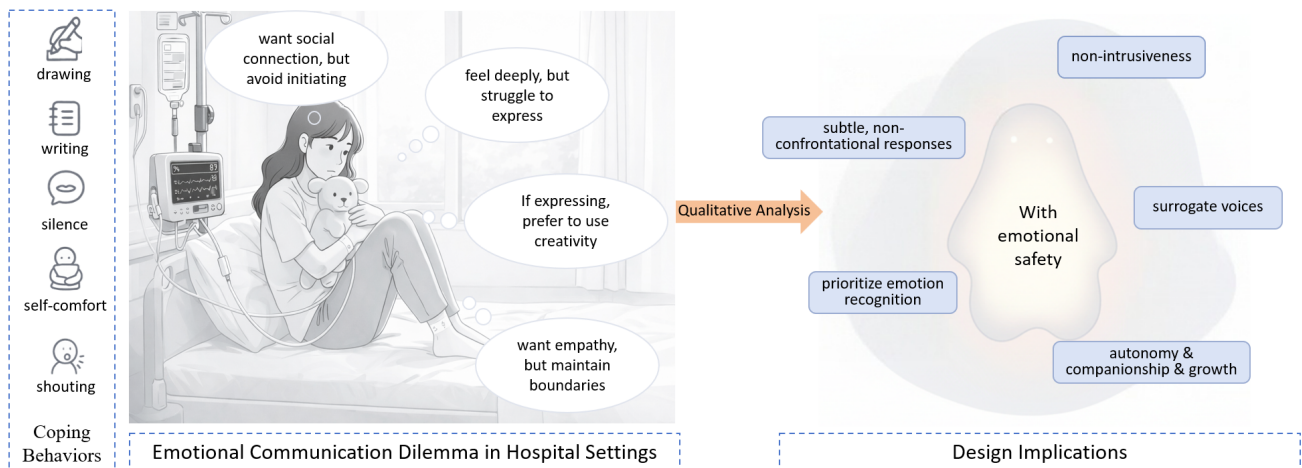


Figure 1: A conceptual overview of hospitalized adolescents' emotional communication dilemma, coping behaviors, and implications for emotional expression tools.

* Corresponding authors

[†] Jin Huang is also with the School of Computer Science and Technology, University of Chinese Academy of Sciences.



This work is licensed under a Creative Commons Attribution 4.0 International License.
CHI EA '26, Barcelona, Spain

© 2026 Copyright held by the owner/author(s).

ACM ISBN 979-8-4007-2281-3/26/04

<https://doi.org/10.1145/3772363.3798447>

Abstract

Hospitalized adolescents often encounter difficulties in emotional communication due to the mismatch between their developmental needs and adult-centered clinical environments. While prior work has explored psychological and technological interventions, less is known about adolescents' own experiences and acceptable forms of support. We present a qualitative study using questionnaires and semi-structured interviews to examine adolescents' emotional communication in hospital settings. Our analysis reveals ambivalent

social connection, indirect and mediated emotional communication, and adolescents' expectations for emotional expression tools. Based on these findings, we discuss implications for designing low-intrusion emotional support tools that align with adolescents' emotional needs in hospital settings.

Keywords

Hospitalized Adolescents, Emotional Communication, Qualitative Methods, Emotional Support Technology

ACM Reference Format:

Liuping Wang, Mingjing Wu, Shengmin Zhao, Wenhui Kang, Jin Huang, Chenglei Yang, Dong Yu, and Pengyuan Liu. 2026. "I Want My Own Space, but I Want to Be Understood": The Adolescent Emotional Communication Dilemma in Hospitals. In *Extended Abstracts of the 2026 CHI Conference on Human Factors in Computing Systems (CHI EA '26)*, April 13–17, 2026, Barcelona, Spain. ACM, New York, NY, USA, 6 pages. <https://doi.org/10.1145/3772363.3798447>

1 Introduction

In hospital settings largely shaped by clinical workflows and professional protocols, adolescents commonly experience difficulties in emotional communication [6, 32]. Prior research has shown that such communication barriers are associated with elevated mental health risks and poorer perceptions of healthcare experiences [8]. During hospitalization, adolescents must simultaneously cope with illness-related stress, disruptions to schooling, separation from family, and psychological needs [13, 30]. However, adult-centered models of patient–provider communication frequently fail to address adolescents' needs for autonomy, emotional expression, and privacy [9], contributing to their tendency to avoid or withhold genuine feelings, particularly when discussing sensitive psychological or social concern [1].

Building on these observations, recent work in healthcare has explored psychological counseling, peer support programs, and digital tools to improve emotional support for adolescents [17, 24]. Nevertheless, current interventions continue to face limitations in accessibility, engagement, personalization, and sustainability [3, 4, 18]. Many technological solutions primarily emphasize information delivery or symptom management [21], while offering more limited support for emotional connection, trust-building, and sustained bidirectional interaction [14, 16, 42, 47]. Moreover, their design processes fail to adequately incorporate adolescents' everyday practices, technology use patterns, and privacy expectations [2, 46], thereby constraining adoption and long-term use. As a result, there remains a lack of systematic understanding of how hospitalized adolescents themselves perceive emotional communication challenges and what forms of support or technological mediation they find acceptable and meaningful [11].

In this study, we focus on understanding hospitalized adolescents' perspectives on emotional communication support through a needs-oriented qualitative inquiry. We collected questionnaire data from 34 adolescents with recent hospitalization experiences and conducted a qualitative analysis to identify recurring barriers, needs, and preferences. Our findings indicate that adolescents experience ambivalent social needs, rely on non-verbal strategies and creative practices for emotional expression and self-regulation,

and favor low-intrusiveness, autonomy-preserving forms of support. Building on these insights, we derive design implications for adolescent-centered emotional communication tools in hospital settings.

2 Related Work

2.1 Role of Technology in Mediating Adolescents' Emotional Support

In healthcare contexts, much prior healthcare technology research involving adolescents has focused on technologies for assessment and monitoring, such as systems for collecting patient-reported experiences [20], anxiety assessment tools [7], psychological assessment instruments [49], and clinically oriented digital tools [38]. These technologies play an important role in identifying symptoms and informing clinical decision-making; however, they primarily emphasize measurement and detection rather than supporting ongoing emotional and communicative needs.

More recently, adolescents are increasingly recognized as a distinct patient group with specific mental health and emotional support needs [19, 39, 51]. Some prior research has explored digital technologies as mediators of adolescents' emotional support, including chatbots [34, 40], conversational agents [26], and social networking websites [5] that facilitate emotional disclosure, offer non-judgmental shared spaces, and support emotion regulation, highlighting the potential of technology to mediate emotional support for adolescents beyond assessment-oriented use. Despite these advances, much of this emotional support-oriented work remains insufficiently grounded in hospital-specific contexts [28, 43]. Hospitalized adolescents experience multiple stressors, while current technologies provide limited support for emotionally sensitive interactions under hospital constraints.

2.2 Key Considerations for Understanding Adolescents' Emotional Support

Adolescents' perspectives on emotional support differ from those of the adults who care for them, particularly during hospitalization, which introduces illness-related uncertainty, physical discomfort, social isolation, disruption to schooling, and challenges to identity development [28, 43]. Although including adolescents' voices in healthcare is increasingly recognized as important, their perspectives are still only occasionally solicited. Environmental constraints such as restricted autonomy, limited peer interaction, boredom, and reduced participation in daily activities [27, 31, 36, 45] further shape emotional experience and willingness to communicate openly.

Existing research has documented adolescents' support needs in areas such as care, information, relationships, and activities [15], as well as clear expectations for respect, transparent medical communication, and stronger emotional and social support [11, 25]. However, comparatively little work has examined how these needs, particularly emotional ones, can be addressed through interactive or technological mechanisms, while also accommodating cultural, contextual, and personal factors such as anonymity, personalization, and access to localized resources [41]. Building on this understanding, prioritizing adolescents' perspectives rather than relying on

caregiver assumptions can ensure more effective emotional support during hospitalization and facilitate smoother post-discharge transitions, including returning to school and reintegration into daily life [34, 37, 48].

3 Methodology

We conducted a qualitative study using questionnaires and semi-structured interviews with 34 hospitalized adolescents (9 males, 25 females; aged 10–18) recruited from a child and adolescent mental health inpatient unit. Participants experienced emotional difficulties requiring inpatient care, such as emotional instability or school-related distress. Our analysis focused on shared social, emotional, and communicative experiences in a semi-closed hospital environment rather than diagnostic differences.

The questionnaire and interview prompts focused on adolescents' emotional expression and regulation [29, 44], social and clinical communication experiences [23, 33, 50], interaction preferences and autonomy [10, 12], and engagement, motivation, and imagination [22, 35] within everyday hospitalization contexts.

Data were collected through questionnaires and semi-structured interviews. Electronic observation logs documented adolescents' behaviors and contextual experiences during hospitalization. All interviews were audio-recorded and transcribed verbatim for analysis. Data analysis followed an open coding approach. Three researchers independently reviewed the data to achieve familiarization and conducted open coding, generating 321 initial codes, which were managed using NVivo. Coding disagreements were resolved through discussion among the three researchers until consensus was reached. Through iterative discussions, axial and selective coding were subsequently applied, resulting in the identification of four core themes.

This study followed local ethics committee guidelines. Written informed consent was obtained from all participants and their parents, and approval was also obtained from the attending physicians prior to participation. Participants' confidentiality and anonymity were strictly maintained.

4 Findings

Our analysis identified four interconnected themes that reflect hospitalized adolescents' emotional communication experiences, including ambivalent social connection and interaction, as shown in Table 1, indirect emotional expression and self-regulation, mediated expression through creative practices, and expectations for emotional expression tools.

4.1 Ambivalent Social Connection and Interaction

Hospitalized adolescents in semi-enclosed environments demonstrated ambivalent social needs: they strongly desired connection with others while also valuing personal space and privacy. Participants hoped to share positive experiences, engage in activities together, and build relationships around shared interests. For example, one participant stated that they *"wanted a space of their own"* while also *"hoping someone would ask me to go out and play together"* (P2).

Despite this desire for connection, adolescents often adopted passive or avoidant interaction strategies. Eight participants explicitly reported hoping that *"others would take the initiative,"* while others described withdrawing behaviors such as staying silent, walking away, or choosing not to participate in shared activities. Several participants emphasized the emotional effort required to initiate communication, noting that approaching others required substantial *"psychological preparation"* (P28).

This ambivalence was further intensified by the clinical context. Limited social resources and the hierarchical nature of patient-provider relationships reduced adolescents' confidence in expressing themselves. Some participants felt that healthcare professionals *"did not understand their feelings"* (P6, P20, P22, P24), while others reported difficulty articulating bodily discomfort or emotional distress, particularly when symptoms were ambiguous. As a result, adolescents expressed interest in alternative communication modalities, such as drawings or emojis, to mediate interactions with clinicians.

Parent-child interactions revealed a similar tension between seeking recognition and maintaining boundaries. Adolescents valued parental attention and positive feedback, particularly praise or acknowledgment of their creative work. At the same time, many explicitly restricted parental access to their expressions, citing fears of criticism or concern about causing parental worry. This selective sharing reflected adolescents' efforts to protect their emotional autonomy while still desiring care and validation.

4.2 Emotional Expression and Self-Regulation

Adolescents' emotional expression in daily interactions was typically immediate, embodied, and non-verbal. When faced with emotional distress, many participants reported difficulty articulating feelings verbally and instead relied on silence, withdrawal, bodily actions, or sensory behaviors to convey their emotional states.

Silence emerged as a dominant form of emotional expression. Several adolescents described becoming quiet when feeling angry or sad, while younger participants more frequently exhibited overt non-verbal behaviors such as crying, shouting, or physical agitation. Writing, drawing, and sound-based behaviors also appeared as alternative outlets when spoken communication felt inadequate, suggesting that non-verbal modalities served as compensatory channels for emotional expression.

Emotion regulation strategies were predominantly internalized. Adolescents predominantly preferred to manage distress alone, avoiding visible expressions of sadness and declining comfort from others. Self-soothing behaviors included withdrawing to private spaces, physical self-comfort such as *"hugging objects"* (P13, P16, P25), or waiting for emotions to pass. However, a small subset of participants described maladaptive regulation strategies when emotional pressure accumulated, including aggressive outbursts or self-directed harm, reflecting a lack of accessible and effective coping resources.

When discussing emotional support, adolescents consistently emphasized the importance of companionship over verbal reassurance. Rather than receiving advice or rational explanations, participants expressed a preference for calm, empathetic presence—such as someone staying nearby, offering gentle physical comfort, or

Table 1: Qualitative examples of present themes

Themes	Quotations	
Ambivalent Social Connection and Interaction Emotional Expression and Self-Regulation Creative Imagination and Expression Expectations for Emotional Expression Tools	<i>"When I feel lonely but surrounded by strangers, I really want to talk to someone, but I often don't know how to start"</i>	<i>"I don't really want others to see me when I'm unhappy, and I'm not sure they would understand. But if someone notices without me trying, I might still have a little hope"</i>
	<i>"When I feel upset or angry, I usually don't want to talk and prefer to process my emotions alone. I am also afraid of taking my negative feelings out on others"</i>	<i>"I used to cry to let others know I was sad, but later realized that crying alone doesn't make them understand what I'm thinking, so I gradually stopped expressing it."</i>
	<i>"I want to express my feelings and inner experiences through drawing, because some emotions are hard to put into words"</i>	<i>"I enjoy writing novels based on myself, like fantasy or martial arts stories. Completing a story gives me a strong sense of accomplishment, and I even like to share it with others"</i>
	<i>"I hope there is a tool that can help me express emotions I cannot articulate and communicate them to others"</i>	<i>"I would prefer it to be a relatively private space rather than something connecting to family; having a space solely for my own emotions is more comfortable"</i>

conveying understanding without judgment. Superficial or didactic forms of comfort were often rejected, as they failed to address adolescents' underlying need to feel seen and emotionally safe.

4.3 Creative Imagination and Expression

In contrast, creative activities provided a more mediated and self-controlled pathway for emotional expression among hospitalized adolescents. Rather than expressing emotions directly through conversation, participants often turned to drawing, writing, or storytelling to externalize feelings in ways that felt safer and more manageable. Creative engagement was frequently triggered by boredom, loneliness, fear, or emotional distress, with several adolescents describing creation as a means to *"express inner feelings"*(P6) when verbal articulation felt difficult.

Beyond emotional release, creative practices also enabled adolescents to construct alternative identities and regain a sense of agency. Participants centered themselves in imagined narratives, portraying empowered or idealized versions of the self, such as heroic figures or world builders. Notably, some adolescents incorporated clinical roles into their stories, imagining themselves as doctors who could *"help others"*(P31) or *"save lives"*(P23), reflecting an attempt to reframe their hospitalized status and assert control within a constrained medical environment.

Adolescents expressed strong expectations regarding autonomy over their creative outputs. Many emphasized their right to decide whether their works remained private or were shared with others, often differentiating between trusted and untrusted audiences. While some participants welcomed public visibility and feedback, others preferred to keep their creations private, underscoring the importance of personal boundaries in creative expression.

At the same time, positive feedback played a critical role in sustaining creative motivation. Adolescents desired encouragement, comfort, or gentle suggestions rather than criticism. Praise and acknowledgment were perceived not merely as responses to the artwork itself, but as validation of the adolescent's emotional state and self-worth.

4.4 Expectations for Emotional Expression Tools

Adolescents articulated clear expectations for tools that could support emotional expression, with emotion recognition and timely, empathetic response identified as the most critical functions. Participants hoped the tool could accurately sense emotional changes and *"understand how I feel,"* emphasizing companionship-oriented interactions such as chatting(P6, P18, P28, P34), playing games together(P7, P13, P17, P34), or offering comforting gestures(P15, P25). Adolescents who struggled with self-expression additionally valued the tool's ability to mediate communication by translating internal feelings into clearer messages for others. These expectations highlight a preference for emotional attunement rather than directive guidance or problem-focused intervention.

Building on this, some adolescents further expected the tool to support emotional reflection by recording daily experiences or emotional fluctuations, enabling them to recognize patterns and better understand their own emotional states. At the same time, engagement should remain gentle and non-intrusive, appearing only when needed and avoiding over-interpretation or forced interaction, reflecting adolescents' sensitivity to autonomy and emotional boundaries.

Preferences regarding appearance and sensory interaction further underscored adolescents' desire for low-pressure engagement. Soft textures, warmth, gentle lighting, and calming sounds were associated with feelings of safety and relaxation, while animal-like forms were favored. Moreover, entertainment and positive feedback, such as lightweight games and rewards, were seen as integral to emotional support, providing companionship, alleviating negative emotions, and reinforcing a sense of progress while supporting communication, reflection, and personal growth.

5 Discussion and Implication

While some of the emotional needs identified, such as preferences for creative expression, may also appear in non-clinical settings, the hospital context intensifies these challenges through restricted autonomy, hierarchical patient-provider dynamics, and limited social resources, making tailored support particularly critical. The

findings point to several interrelated implications, which can inform strategies to better understand and support hospitalized adolescents' emotional communication, all within the framework of ensuring emotional safety:

- Adolescents' ambivalent desire for social connection underscores the need for emotional support that is available **without being intrusive**. Rather than encouraging constant engagement, emotional support systems should remain passively accessible, allowing adolescents to decide when interaction occurs, aligning with their need for personal space and emotional safety. Compared to human-led interventions, digital tools offer unique affordances such as anonymity, asynchronous interaction, and user-controlled engagement, which may lower the threshold for emotional disclosure in clinical settings.
- Adolescents' tendency to suppress or internalize emotions suggests that emotional support should **prioritize emotional recognition** over direct questioning. When direct verbal expression feels difficult or unsafe, support that acknowledges emotional states through **subtle, non-confrontational responses** may be more acceptable than repeated prompts or inquiries. However, low-intrusion support must be bounded by appropriate safety mechanisms; when indicators of acute distress or self-harm risk are detected, escalation pathways to clinical professionals should be activated.
- Communication barriers with peers, caregivers, and health-care professionals highlight the role of tools to function as **"surrogate voices"** for emotional expression. By helping adolescents externalize emotions and articulate experiences in structured or mediated ways, such tools can ease the communicative burden without requiring adolescents to initiate or sustain direct communication themselves. For example, a tool might allow adolescents to select visual symbols, emojis, or pre-composed emotional expressions to convey their current state to clinicians, reducing the burden of verbal articulation while preserving communicative intent.
- Adolescents' expectations for emotional expression tools further emphasize **companionship, autonomy, and growth** over directive intervention. Tools that offer gentle presence, optional engagement, respect their control over use and disclosure, and positive reinforcement may function not as authoritative systems, but as flexible companions that adapt to adolescents' emotional rhythms over time.

6 Future Work and Conclusion

This study examined hospitalized adolescents' experiences of emotional communication through questionnaires and interviews. The findings highlight ambivalent social connection, indirect and mediated emotional expression, and adolescents' preference for companionship-oriented support that respects autonomy and personal boundaries. This study has several limitations. First, the gender distribution of our sample was unbalanced (25 females, 9 males), and the wide age range (10–18) may encompass developmentally diverse subgroups whose experiences differ; future research should explore potential gender- and age-related variations. Second, all

participants were recruited from a mental health inpatient unit, which may limit the generalizability of findings to adolescents hospitalized for other conditions. Third, the cross-sectional design does not capture changes in emotional communication patterns before and during hospitalization. Future work will focus on designing and evaluating emotional expression tools that align with these needs in hospital settings.

References

- [1] Dennis Adjei-Boateng et al. 2025. Digital mental health interventions for adolescents and young people: evaluating efficacy and accessibility. *Cureus* 17, 6 (2025).
- [2] Holly Agostino and Brett Burstein. 2023. Perceived barriers to the provision of adolescent confidential care in a tertiary care setting. *Paediatrics & Child Health* 28, 2 (2023), 91–96.
- [3] Holly Alice Bear, Lara Ayala Nunes, Giovanni Ramos, Tanya Manchanda, Blossom Fernandes, Sophia Chabursky, Sabine Walper, Edward Watkins, and Mina Fazel. 2024. The acceptability, engagement, and feasibility of mental health apps for marginalized and underserved young people: systematic review and qualitative study. *Journal of Medical Internet Research* 26 (2024), e48964.
- [4] Patricia Berkanish, Samuel Pan, Adrienne Viola, Quinn Rademaker, and Katie A Devine. 2022. Technology-based peer support interventions for adolescents with chronic illness: a systematic review. *Journal of clinical psychology in medical settings* 29, 4 (2022), 911–942.
- [5] Camila Amaral Borghi, Regina Szytli, Caroliny Rossi de Faria Ichikawa, Michelle Freire Baliza, Uyara Talmatare Jesus Camara, and Heloisa Cristina Figueiredo Frizzo. 2018. Use of social networking websites as a care instrument for hospitalized adolescents. *Escola Anna Nery* 22, 1 (2018), e20170159.
- [6] Cora C Breuner, Elizabeth M Alderman, and Jennifer A Jewell. 2023. The hospitalized adolescent. *Pediatrics* 151, 2 (2023), e2022060647.
- [7] Fernanda Vieira de Campos, Camila Forni Antunes, Elaine Buchhorn Cintra Damião, Lisabelle Mariano Rossato, and Lucila Castanheira Nascimento. 2020. Anxiety assessment tools in hospitalized children. *Acta paulista de enfermagem* 33 (2020), eAPE20180250.
- [8] A Chen, C Dinyarian, F Inglis, C Chiasson, and Kristin Cleverley. 2022. Discharge interventions from inpatient child and adolescent mental health care: a scoping review. *European child & adolescent psychiatry* 31, 6 (2022), 857–878.
- [9] Alon Coret, Dennis Newhook, and Megan E Harrison. 2024. Adolescents on an inpatient unit and their healthcare providers: what's working and what's not. *International Journal of Adolescent Medicine and Health* 36, 1 (2024), 45–53.
- [10] Christopher Daddis. 2011. Desire for increased autonomy and adolescents' perceptions of peer autonomy: "Everyone else can; why can't I?". *Child development* 82, 4 (2011), 1310–1326.
- [11] Roberta De Rosa, Maria Anna Siano, Angelo Colucci, Anna Giulia Elena De Anseris, Paolo Siani, Pietro Vajro, Giulia Savarese, and Claudia Mandato. 2024. Perceptions and expectations of youth regarding the respect for their rights in the hospital. *Children* 11, 2 (2024), 222.
- [12] Vikki A Entwistle, Stacy M Carter, Alan Cribb, and Kirsten McCaffery. 2010. Supporting patient autonomy: the importance of clinician-patient relationships. *Journal of general internal medicine* 25, 7 (2010), 741–745.
- [13] Mina Fazel, Emma Soneson, Michael Parker, Danielle Newby, and Jonathan Herring. 2025. Adolescent consent and Generation Alpha: bridging policy, practice and empirical evidence in healthcare. *The British Journal of Psychiatry* (2025), 1–6.
- [14] Stuart W Grande, Meghan R Longacre, Karin Palmblad, Meera V Montan, Rikard P Berquist, Andreas Hager, and Greg Kotzbauer. 2019. Empowering young people living with juvenile idiopathic arthritis to better communicate with families and care teams: content analysis of semistructured interviews. *JMIR mHealth and uHealth* 7, 2 (2019), e10401.
- [15] Laila Hajitagh, Ali Soleimani, and Parvaneh Aghajari. 2025. Hospitalized Children's Needs in Their Own Words: A Self-Report Study. *International Journal of Africa Nursing Sciences* 23 (2025), 100915.
- [16] Heather K Hardin, Anna E Bender, Carla P Hermann, and Barbara J Speck. 2021. An integrative review of adolescent trust in the healthcare provider relationship. *Journal of Advanced Nursing* 77, 4 (2021), 1645–1655.
- [17] Gemma Heath, Hermione Montgomery, Caron Eyre, Carole Cummins, Helen Pattison, and Rachel Shaw. 2016. Developing a tool to support communication of parental concerns when a child is in hospital. *Healthcare* 4, 1 (13 Jan. 2016). doi:10.3390/healthcare4010009 Funding Information: Acknowledgments: This project was funded by the Health Foundation as part of its SHINE funding stream. Gemma Heath and Carole Cummins were funded by the National Institute for Health Research (NIHR) through the Collaborations for Leadership in Applied Health Research and Care for Birmingham and Black Country (CLAHRC-BBC)

- programme. This paper presents independent research partly funded by the National Institute for Health Research.
- [18] Matthew K. Hong, Udaya Lakshmi, Kimberly Do, Sampath Prahalad, Thomas Olson, Rosa I. Arriaga, and Lauren Wilcox. 2020. Using Diaries to Probe the Illness Experiences of Adolescent Patients and Parental Caregivers. In *Proceedings of the 2020 CHI Conference on Human Factors in Computing Systems* (Honolulu, HI, USA) (CHI '20). Association for Computing Machinery, New York, NY, USA, 1–16. doi:10.1145/3313831.3376426
 - [19] Min-Wei Hung, Chien Wen Yuan, Nanyi Bi, Yi-Chao Chen, Wan-Chen Lee, Ming-Chyi Huang, and Chuang-Wen You. 2022. To use or abuse: opportunities and difficulties in the use of multi-channel support to reduce technology abuse by adolescents. *Proceedings Of The Acm On Human-Computer Interaction* 6, CSCW1 (2022), 1–27.
 - [20] Jane Hybschmann, Jette Led Sørensen, Jakob Thestrup, Helle Pappot, Kirsten Arntz Boisen, Thomas Leth Frandsen, and Line Klinge Gjærde. 2024. MyHospitalVoice—a digital tool co-created with children and adolescents that captures patient-reported experience measures: a study protocol. *Research Involvement and Engagement* 10, 1 (2024), 49.
 - [21] Margot Jager, Andrea F De Winter, Janneke Metselaar, Erik J Knorth, and Sijmen A Reijneveld. 2015. Priorities in the communication needs of adolescents with psychosocial problems and their parents. *Health Expectations* 18, 6 (2015), 2811–2824.
 - [22] Margot Jager, Sijmen A Reijneveld, Janneke Metselaar, Erik J Knorth, and Andrea F De Winter. 2014. Discrepancies between adolescents' attributed relevance and experiences regarding communication are associated with poorer client participation and learning processes in psychosocial care. *Patient education and counseling* 97, 3 (2014), 332–338.
 - [23] Matthias G Klosinski and Erik Farin. 2015. Communication preferences of chronically ill adolescents: Development of an assessment instrument. *Psychological Assessment* 27, 3 (2015), 1053.
 - [24] Theodora Koulouri, Robert D Macredie, and David Olakitan. 2022. Chatbots to support young adults' mental health: an exploratory study of acceptability. *ACM Transactions on Interactive Intelligent Systems (TiIS)* 12, 2 (2022), 1–39.
 - [25] Ila K. Kumar, Craig Ferguson, Jiayi Wu, and Rosalind W. Picard. 2025. Cultivating a Supportive Sphere: Designing Technology to Increase Social Support for Foster-Involved Youth. *Proc. ACM Hum.-Comput. Interact.* 9, 2, Article CSCW054 (May 2025), 24 pages. doi:10.1145/3710952
 - [26] Irene Lopatovska and Jessika Davis. 2023. Designing Supportive Conversational Agents With and For Teens. In *Proceedings of the 2023 Conference on Human Information Interaction and Retrieval*. 328–332.
 - [27] Irene Lopatovska, Olivia Turpin, Jessika Davis, Ellen Connell, Chris Denney, Hilda Fournier, Archana Ravi, Ji Hee Yoon, and Eesha Parasnis. 2022. Capturing teens' voice in designing supportive agents. In *Proceedings of the 4th Conference on Conversational User Interfaces*. 1–12.
 - [28] Dorit Maor and Katherine Mitchem. 2020. Hospitalized adolescents' use of mobile technologies for learning, communication, and well-being. *Journal of Adolescent Research* 35, 2 (2020), 225–247.
 - [29] Jennifer R McCullen, Scott P Mirabile, Shelby M Wuertz, and Brandon G Scott. 2024. Development and adaptation of the Emotion Regulation Skills Questionnaire for adolescents. *Journal of Adolescence* 96, 3 (2024), 632–644.
 - [30] Vanessa McFadden, Kelsey Porada, Nicole Emlen, Michelle L Pickett, and Katherine Quinn. 2024. Hospitalized Adolescents' Perspectives on Sexual and Reproductive Health Discussions. *Hospital Pediatrics* 14, 7 (2024), 548–555.
 - [31] Sian McKenzie, Sara Norrish, Leah Parker, and Ian Frampton. 2010. Consulting with young people about healthcare. Part I: experience of the hospital environment. *Pediatric Health* 4, 2 (2010), 157–166.
 - [32] Alice Navein, James McTaggart, Xanthe Hodgson, Joanna Shaw, Dougal Hargreaves, Eva Gonzalez-Viana, and Agnesa Mehmeti. 2022. Effective healthcare communication with children and young people: a systematic review of barriers and facilitators. *Archives of disease in childhood* 107, 12 (2022), 1111–1116.
 - [33] Faezeh Nehzandoost, Ali Vafaei Najar, Marziye Zarqi, Jamshid Jamali, Seyed Saeed Tabatabaei, and Elaheh Hooshmand. 2025. Development and validation of a patient experience assessment questionnaire for evaluating hospital care in Iran. *BMC Health Services Research* 25, 1 (2025), 62.
 - [34] Yeming Ni, Ruyi Ding, Yuqing Chen, Hanchao Hou, and Shiguang Ni. 2023. Focusing on Needs: A Chatbot-Based Emotion Regulation Tool for Adolescents. In *2023 IEEE International Conference on Systems, Man, and Cybernetics (SMC)*. IEEE, 2295–2300.
 - [35] Atefeh Omrani, Joanna Wakefield-Scurr, Jenny Smith, and Nicola Brown. 2019. Survey development for adolescents aged 11–16 years: A developmental science based guide. *Adolescent Research Review* 4, 4 (2019), 329–340.
 - [36] Kimberly Peeters, Pleuntje Jellema, Margo Annemans, and Ann Heylighen. 2018. How do adolescents affected by cancer experience a hospital environment? *Journal of Adolescent and Young Adult Oncology* 7, 4 (2018), 488–492.
 - [37] Michèle Preyde, Shrenik Parekh, Amanda Warne, and John Heintzman. 2017. School reintegration and perceived needs: The perspectives of child and adolescent patients during psychiatric hospitalization. *Child and Adolescent Social Work Journal* 34, 6 (2017), 517–526.
 - [38] Emanuele Rinninella, Antonio Ruggiero, Palma Maurizi, Silvia Triarico, Marco Cintoni, and Maria Cristina Mele. 2017. Clinical tools to assess nutritional risk and malnutrition in hospitalized children and adolescents. *European Review for Medical & Pharmacological Sciences* 21, 11 (2017).
 - [39] Jami-Leigh Sawyer, Faye Mishna, Eric Bouffet, Michael Saini, and Randi Zlotnik-Shaul. 2023. Bridging the gap: Exploring the impact of hospital isolation on peer relationships among children and adolescents with a malignant brain tumor. *Child and Adolescent Social Work Journal* 40, 1 (2023), 91–105.
 - [40] Neil KR Sehgal, Hita Kambhamettu, Sai Preethi Matam, Lyle Ungar, and Sharath Chandra Guntuku. 2025. Designing Mental-Health Chatbots for Indian Adolescents: Mixed-Methods Evidence, a Boundary-Object Lens, and a Design-Tensions Framework. *arXiv preprint arXiv:2511.07729* (2025).
 - [41] Neil KR Sehgal, Hita Kambhamettu, Sai Preethi Matam, Lyle Ungar, and Sharath Chandra Guntuku. 2025. Exploring Socio-Cultural challenges and opportunities in designing mental health chatbots for adolescents in India. In *Proceedings of the Extended Abstracts of the CHI Conference on Human Factors in Computing Systems*. 1–7.
 - [42] Soyoung Shin and Suyoun Ahn. 2023. Experience of adolescents in mental health inpatient units: A metasynthesis of qualitative evidence. *Journal of psychiatric and mental health nursing* 30, 1 (2023), 8–20.
 - [43] Ana M Ullán, Manuel H Belver, Isabel Serrano, Juan Delgado, and Marta Badía. 2012. Perspectives of youths and adults improve the care of hospitalized adolescents in Spain. *Journal of Pediatric Health Care* 26, 3 (2012), 182–192.
 - [44] Daoyang Wang, Bin Yuan, Hui Han, and Cuicui Wang. 2022. Validity and reliability of emotion regulation questionnaire (ERQ) in Chinese rural-to-urban migrant adolescents and young adults. *Current Psychology* 41, 4 (2022), 2346–2353.
 - [45] Hui Wang, Jianjie Xu, Sinan Fu, Ue Ki Tsang, Haining Ren, Shurou Zhang, Yueqin Hu, Janice L Zeman, and Zhuo Rachel Han. 2024. Friend emotional support and dynamics of adolescent socioemotional problems. *Journal of youth and adolescence* 53, 12 (2024), 2732–2745.
 - [46] Lu Wang, Haiou Zou, Yuan Yang, and Jingfang Hong. 2023. Adolescents' attitudes toward non-suicidal self-injury (NSSI) and their perspectives of barriers to seeking professional treatment for NSSI. *Archives of psychiatric nursing* 45 (2023), 26–35.
 - [47] Emily Weinstein, Evan M Kleiman, Peter J Franz, Victoria W Joyce, Carol C Nash, Ralph J Buonopane, and Matthew K Nock. 2021. Positive and negative uses of social media among adolescents hospitalized for suicidal behavior. *Journal of Adolescence* 87 (2021), 63–73.
 - [48] Ling Wu, Joshua Paolo Seguin, Dharshani Chandrasekara, Mairead Claire Cardamone-Breen, Jue Xie, Roisin Mcnane, Tom Bartindale, Patrick Olivier, and Marie BH Yap. 2024. Designing online peer support for parents of adolescents at risk of mental health challenges. In *Proceedings of the 2024 CHI Conference on Human Factors in Computing Systems*. 1–14.
 - [49] Lorena F Zhang, Jane SW Ho, and Sean E Kennedy. 2014. A systematic review of the psychometric properties of transition readiness assessment tools in adolescents with chronic disease. *BMC pediatrics* 14, 1 (2014), 4.
 - [50] Rui Zhen, Lu Li, Guoqiang Li, and Xiao Zhou. 2023. Social isolation, loneliness, and mobile phone dependence among adolescents during the COVID-19 pandemic: roles of parent-child communication patterns. *International Journal of Mental Health and Addiction* 21, 3 (2023), 1931–1945.
 - [51] JunTao Zheng and JiaYi Chen. 2025. Mobile Technology for Adolescent Mental Health: Designing and Evaluating an Online Emotional Support System in Low-Income Communities. *International Theory and Practice in Humanities and Social Sciences* 2, 2 (2025), 1–21. doi:10.70693/itphss.v2i2.202